

CANONICAL LESSON

V7_B_N13 - Emergency Operations Decision Room

Decision-centred foundations for accountable sector intelligence

Production Unit	PU-B07-C33
Notebook	V7_B_N13
Status	Controlled educational draft v0.1
Data boundary	Synthetic instructional data only

Central decision: Operate a health emergency decision room with verified evidence, uncertainty, capacity, authority, escalation, and closure.

Learning outcomes

Why this matters

A fast dashboard can amplify unverified signals and obscure operational authority.

The analytical workflow is useful only when evidence, uncertainty, authority, and action are connected explicitly. The model supports professional judgement; it does not replace accountable institutional authority.

Worked cross-sector example

An incident brief links severity, resources, responsible owners, review times, and escalation paths.

Boundary: The example supports learning and review. It does not constitute an official statistic, eligibility determination, enforcement action, clinical judgement, or operational authorization.

Practice and reflection

- 1.
- Deduplicate incident reports.
- 2.
- Add asymmetric escalation costs.
- 3.

- Protect the incident log.

Answer key

- 1.
- Use stable event keys, time-location windows, provenance, and human reconciliation.
- 2.
- Make missed severe events costlier while controlling alarm burden and capacity.
- 3.
- Minimize personal data, restrict access, preserve revisions, record decisions, and review after the event.

Operational readiness checklist

- Purpose and decision owner are named.
- Evidence definitions, provenance, and lawful access are documented.
- Uncertainty and abstention are visible.
- Human review, escalation, monitoring, incident response, and retirement are assigned.
- Equity, accessibility, security, and sector-specific approval are complete.